

C. Matthew Duncun's method

D. Smellie-Veit technique

Answer: C. Matthew Duncan's method

Rationale:

Matthew Duncan relates to placental delivery, not breech delivery.

18. On per vaginal examination, anterior fontanelle and supra orbital ridge are felt in the second stage of labor. The presentation is

A. Brow presentation

B. Deflexed head

C. Face presentation

D. Malpresentation

Answer: A. Brow presentation

Rationale:

Brow presentation lies between vertex and face; these landmarks are classic.

19. A multigravida with previous 12 normal deliveries presents with unstable lie of the fetus at 34 weeks gestation. What could be the most probable cause?

A. Oligohydramnios

B. Pelvic tumor

C. Placenta previa

D. Uterine anomaly

Answer: C. Placenta previa

Rationale:

Placenta previa prevents engagement of the presenting part, causing unstable lie.

20. Which of the following presentations has the largest engaging diameter?

A. Breech

B. Brow

C. Face

D. Vertex

Answer: B. Brow

Rationale:

Brow presents the mento-vertical diameter, the largest possible fetal diameter.

21. A 26 year old female in the labour room, when the physician performs a vaginal examination he felt the one foot of the baby. What is the type of presentation?

A. Complete breech

B. Frank breech

C. Incomplete breech

D. Vertex

Answer: C. Incomplete breech

Rationale:

Footling breech is an incomplete breech presentation.

22. Which of the following correctly describes the presentation of the fetus?

- A. Fetal part that first enters the maternal pelvis
- B. Relationship between the transverse axis of the fetus and mother
- C. Orientation of the fetal head as it exits the birth canal
- D. Most superior part of the fetus in the maternal pelvis

Answer: A. Fetal part that first enters the maternal pelvis

Rationale:

Presentation refers specifically to the leading fetal part.

23. Patient with shoulder presentation is examined by examiner at ANC clinic. Most appropriate investigation needed is

- A. Down syndrome screening
- B. External cephalic version
- C. Internal podalic version

D. Ultrasound scan

Answer: D. Ultrasound scan

Rationale:

Ultrasound confirms lie, presentation, and placental location.

24. A type of breech where one or both legs are extended at the hip and knee joint so that the baby presents foot first is

A. Complete breech

B. Frank breech

C. Footling breech

D. None of the above

Answer: C. Footling breech

Rationale:

Legs extended → feet present first.

25. Which of the following is an unfavorable factor for anterior long rotation of the occiput in case of occipito posterior position?

- A. Average size of fetal head
- B. Gynecoid pelvis
- C. Premature rupture of membranes
- D. Regular uterine contractions

Answer: C. Premature rupture of membranes

Rationale:

Loss of liquor reduces lubrication and fetal rotation.

26. What is the most common fetal malposition?

- A. Breech
- B. Brow
- C. Face
- D. Vertex

Answer: A. Breech

Rationale:

Breech is the most common abnormal presentation.

27. A 30 years old G1P0 client in labour with transverse lie. Management choice includes:

- A. Emergency cesarean section
- B. External cephalic version
- C. Internal cephalic version
- D. Wait and observe

Answer: A. Emergency cesarean section

Rationale:

Vaginal delivery is impossible in established transverse lie.

28. Why is maternal pyrexia an important complication during the first stage of labour?

- A. May cause maternal exhaustion and oliguria

- B. May lead to convulsions
- C. May cause hypertension during labour
- D. May be a sign of infection

Answer: D. May be a sign of infection

Rationale:

Fever suggests chorioamnionitis, a serious complication.

29. Which of the following may cause hypertension during labour?

- A. Anaemia
- B. Anxiety
- C. Chorioamnionitis
- D. Foetal distress

Answer: B. Anxiety

Rationale:

Anxiety increases sympathetic activity → raised BP.

30. A sign of maternal exhaustion during labour among these is

- A. Bradycardia
- B. Dry mouth and oliguria
- C. Pallor and hypotension
- D. Proteinuria

Answer: B. Dry mouth and oliguria

Rationale:

Indicates dehydration and poor renal perfusion.

31. What may cause maternal exhaustion during labour?

- A. Chorioamnionitis
- B. Placenta praevia

C. Preterm labour

D. Prolonged labour

Answer: D. Prolonged labour

Rationale:

Long labour depletes maternal energy and fluids.

32. How should you treat a patient with maternal exhaustion?

A. Deliver the infant by cesarean section

B. Give 2 litres of Ringer's with 5% dextrose IV

C. Give oxygen by face mask

D. Stop the contractions with tocolytics

Answer: B. Give 2 litres of Ringer's with 5% dextrose IV

Rationale:

Corrects dehydration and restores energy.

33. Labour is said to be prolonged when it exceeds

A. 8 to 10 hours

B. 14 to 20 hours

C. 18 to 24 hours

D. 20 to 36 hours

Answer: C. 18 to 24 hours

Rationale:

Standard definition of prolonged labour.